

PHARMACOVIGILANCE INTERNSHIP

TASK 5

INDIVIDUAL CASE SAFETY REPORT (ICSR) Integrated ADR Identification, Reporting, Causality & Classification

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Role: Pharmacovigilance Intern

Case status: Fictional educational training case

Task purpose

This portfolio simulates the work of a Drug Safety Associate completing an end-to-end individual case safety assessment. It integrates the skills demonstrated in Tasks 1-4: ADR identification, structured reporting, causality assessment, and seriousness/expectedness classification.

Continuity rule

The core case is carried forward from the earlier Yogesh Kumar portfolio. The report intentionally does not add unsupported dates, laboratory values, patient identifiers, reporter contact details or other facts that were not supplied.

Important declaration

This is a fictional educational exercise. It is not a real ICSR, patient record, medical advice, or regulatory submission.

1. Executive case summary

Field	Recorded information
Patient	35-year-old female; fictional case; patient initials not provided.
Background	Intermittent migraine; seasonal allergic symptoms.
Concomitant medicine	Cetirizine 10 mg once daily; ongoing before the suspected event.
Suspected medicine	Ibuprofen 400 mg, three times daily after food.
Indication	Bacterial pharyngitis.
Exposure timeline	Day 0: ibuprofen initiated. Day 2: suspected reaction and withdrawal.
Reaction	Pruritic raised rash over both forearms and upper chest with mild swelling around the lips.
Dechallenge	Symptoms began improving within approximately 24 hours after withdrawal.
Rechallenge	Not performed.
Alternative new exposures	No new food, cosmetic, insect exposure or other newly started medicine reported.
Investigations	Not provided.
Reporter/contact	Educational project preparer: Yogesh Kumar. Contact details not provided.

Case status

Suspected ADR: ibuprofen-associated hypersensitivity-type reaction.

Causality: Probable / Likely (Task 3 educational framework).

Seriousness: Non-serious (Task 4 training classification).

Expectedness: Expected for the exercise; actual product RSI verification required in real work.

2. Source-to-case continuity across Tasks 1-4

Task	Primary purpose	Facts carried into Task 5
Task 1	ADR identification and preliminary reasoning	35-year-old female; cetirizine background; ibuprofen new exposure; Day 2 rash + mild lip swelling; improvement after withdrawal; no rechallenge.
Task 2	Structured ADR reporting/documentation	Relative Day 0/Day 2 timeline; missing identifiers, weight, investigations and reporter contact remain unavailable.
Task 3	Causality assessment	Temporal relationship compatible; dechallenge supportive; no rechallenge; no clear competing new exposure; Probable / Likely.
Task 4	Seriousness/expectedness	Non-serious; expected for training, subject to current RSI verification.
Task 5	Integrated ICSR simulation	Combines the four layers without inventing missing data.

Data-integrity principle

The earlier reporting work explicitly marked unavailable information as not provided. Task 5 keeps that discipline because an apparently complete case is not more accurate when gaps are filled with guesses.

Integrated case logic

Identify the event -> document the exposure and chronology -> assess causality -> classify seriousness/expectedness
-> perform quality control -> identify follow-up needs -> close the educational case.

3. Patient information section of the simulated ICSR

Field	Entry	Data-quality note
Patient identifier	Not provided	Use an approved non-identifying identifier in real work.
Age	35 years	Source case.
Sex	Female	Source case.
Weight	Not provided	Do not infer.
Relevant history	Intermittent migraine; seasonal allergic symptoms	Source case.
Pregnancy status	Not provided	Do not assume.
Allergy history	Not provided	Absence of documentation is not equivalent to no allergy.
Other relevant history	Not provided	Needs authorized source verification.

Patient-data quality assessment

The available demographics support the educational assessment, but a complete real ICSR would require additional verified source data. Missing fields are explicitly retained as missing.

Privacy principle

No patient name or direct identifier is invented. The simulated record focuses on safety-relevant facts rather than identifiable personal information.

4. Suspected medicinal product section

Field	Entry	Assessment note
Medicine	Ibuprofen	Newly initiated medicine.
Strength	400 mg	Source case.
Frequency	Three times daily	Source case.
Instruction	After food	Source case.
Route	Not explicitly stated	Verify; do not infer.
Indication	Bacterial pharyngitis	Source case.
Start	Day 0	Relative time only.
Stop	Day 2	Begins dechallenge.
Batch/lot	Not provided	Verify if relevant/available.
Manufacturer/product	Not provided	Verify if available.
Concomitant medicine	Cetirizine 10 mg once daily	Ongoing background medicine.

Medication-history quality

Ibuprofen is the leading suspect because it is the new exposure immediately preceding the reaction. Cetirizine remains visible as concomitant therapy and should not be silently removed from the history.

5. Adverse reaction section

Field	Recorded information
Event term	Pruritic raised rash with mild swelling around the lips.
Location	Both forearms and upper chest; lips involved.
Onset	Day 2 after ibuprofen initiation.
Action taken	Ibuprofen discontinued.
Dechallenge	Symptoms begin improving within approximately 24 hours.
Outcome	Reaction subsequently settles.
Rechallenge	Not performed.
Seriousness	Non-serious in Task 4 educational classification.
Clinical severity	Not formally graded in source; educational characterization is mild-to-moderate symptom burden without documented systemic compromise.

Observed facts vs interpretation

Observed: rash, itching, mild lip swelling, Day 2 onset, withdrawal, improvement, no rechallenge.

Interpretation: hypersensitivity-type ADR pattern; ibuprofen leading suspected medicine.

Seriousness boundary

No airway/breathing/circulatory compromise, hospitalization, disability or other qualifying serious outcome is documented. The Task 4 training classification therefore remains non-serious.

6. ICSR-ready case narrative

A 35-year-old female with intermittent migraine and seasonal allergic symptoms was receiving cetirizine 10 mg once daily. Ibuprofen 400 mg three times daily after food was newly initiated for bacterial pharyngitis. On Day 2, the patient developed an itchy, raised rash involving both forearms and the upper chest, with mild swelling around the lips. No new food, cosmetic product, insect exposure or other newly started medicine was reported. Ibuprofen was discontinued on Day 2. Symptoms began improving within approximately 24 hours and subsequently settled. No rechallenge was performed. Exact calendar dates, weight, laboratory results, patient initials and reporter contact details were not supplied.

Narrative quality checks

Check	Status	Rationale
Chronology clear	Complete	Day 0 -> Day 2 -> withdrawal -> improvement.
Suspect medicine clear	Complete	Ibuprofen identified as new exposure.
Reaction clear	Complete	Rash, itching and lip swelling described.
Outcome clear	Complete	Reaction subsequently settles.
Rechallenge	Complete	Not performed.
Alternative new exposures	Complete	None reported.
Missing data	Complete	Unavailable fields remain unavailable.

Why the narrative avoids overstatement

The narrative uses suspected-ADR language rather than stating definitive causation, consistent with the Probable / Likely assessment.

7. Chronology and temporal relationship

Time	Exposure / event	Safety relevance
Day 0	Ibuprofen 400 mg TID after food initiated for bacterial pharyngitis.	New exposure is the primary temporal anchor.
Day 1	No reaction detail supplied.	Do not infer absence.
Day 2	Pruritic raised rash and mild lip swelling.	Reaction follows recent initiation.
Day 2	Ibuprofen discontinued.	Dechallenge observation begins.
~24 h later	Itching and rash begin improving.	Positive dechallenge signal.
Later	Reaction settles; no rechallenge.	Outcome documented; no re-exposure evidence.

Temporal assessment

Onset about two days after a new ibuprofen exposure is compatible with a suspected medicine-event relationship. Timing alone does not prove causality.

Calendar-date limitation

The source uses relative timing only. A real ICSR should replace Day 0/Day 2 with verified dates if an authorized source provides them; the educational case must not be back-filled with invented dates.

8. Causality assessment - integrated Task 3 review

Factor	Case evidence	Interpretation
Temporal relationship	Reaction on Day 2 after new ibuprofen exposure.	Supports association.
Dechallenge	Symptoms improve within about 24 hours after withdrawal.	Supports suspected relationship.
Rechallenge	Not performed.	No re-exposure evidence.
Alternative causes	No new food, cosmetic, insect exposure or other newly started medicine.	No clear competing new trigger documented.
Concomitant medicine	Cetirizine was already ongoing.	Relevant background therapy.
Reaction pattern	Rash + itching + mild lip swelling.	Compatible with hypersensitivity-type presentation.
Overall	Multiple supportive elements; no rechallenge.	Probable / Likely educational conclusion.

WHO-UMC-style conclusion

Probable / Likely. Reasonable temporal relationship, clinically compatible positive dechallenge and no clear competing new exposure support the association. Rechallenge is absent but is not required for the Probable / Likely category.

Naranjo note

A numerical Naranjo score is not assigned because the full questionnaire evidence needed to score every item was not supplied. A score should come from actual case answers, not from the desired conclusion.

9. Seriousness, severity and expectedness - integrated Task 4 review

Dimension	Finding	Reasoning
Seriousness	Non-serious	No death, life-threatening event, hospitalization, disability/incapacity, congenital anomaly or other qualifying outcome documented.
Clinical severity	Mild-to-moderate educational characterization	Symptoms without documented systemic compromise; not an official regulatory severity code.
Expectedness	Expected for training exercise	Training classification; current product RSI must be verified in real work.
Combined classification	Non-serious / Expected	Both dimensions assessed independently before combination.

Critical distinction

Severity describes intensity/clinical burden. **Seriousness** is a regulatory outcome-based concept. A case can have notable symptoms without meeting serious criteria, while a less dramatic event can be serious if it causes hospitalization or disability.

Expectedness limitation

The supplied case does not include a current product label or company core safety document. Therefore, the Expected classification is retained as a training result, not presented as a verified regulatory determination.

10. Reporter and source-information section

Reporter field	Educational entry	Real-case requirement
Prepared by	Yogesh Kumar	Authorized reporter/source details.
Role	Pharmacovigilance Intern	Qualified reporter/source role as applicable.
Organization	Internship training portfolio	Actual reporting organization/site.
Reporter contact	Not provided	Complete when required from authorized source.
Signature	Not applicable to this PDF simulation	Use applicable real workflow.
Case source	Fictional training scenario derived from Tasks 1-4	Document traceable source in real system.

Source-data confidence

The case is internally consistent with the earlier educational portfolio, but it is not a verified clinical source. The document therefore remains clearly labeled as a training ICSR simulation.

Open data fields

Patient identifier/initials, weight, exact dates, route, labs, product/manufacture identifiers, reporter contact information and other source-specific fields remain not provided.

11. Medical review and differential considerations

Review area	Case observation	Documentation consequence
Reaction phenotype	Pruritic raised rash + mild lip swelling.	Record manifestations; avoid unsupported diagnosis.
Temporal anchor	Day 2 after new ibuprofen exposure.	Supports association.
Alternative exposure review	No new food/cosmetic/insect/medicine.	Weakens one group of competing explanations, but does not exclude all alternatives.
Concomitant therapy	Cetirizine ongoing.	Retain in medication history.
Management	Ibuprofen discontinued; no additional treatment documented in Task 2 source.	Do not invent treatment.
Outcome	Improvement within ~24 h; later settles.	Supports dechallenge.
Rechallenge	Not performed.	No re-exposure evidence.

Clinical reasoning boundary

The case is compatible with a hypersensitivity-type ADR pattern, but the report avoids unsupported diagnostic labels such as anaphylaxis because those were not established in the source case.

Follow-up data that could strengthen the case

Exact dates, route/formulation, weight, previous allergy history, examination findings, investigations, treatment details, complete medication history, outcome details and verified reporter/source information.

12. MedDRA-oriented event documentation (educational)

A real ICSR would use the organization's validated MedDRA coding process. The current case describes multiple manifestations, so the final coding should be selected by a trained coder using the most precise supported concepts.

Observed wording	Potential coding direction	Caution
Itchy / pruritic	Pruritus-related concept	Select current-version LLT/PT using validated tools.
Raised rash	Rash/eruption-related concept	Do not select a specific dermatologic diagnosis not documented.
Mild lip swelling	Lip/facial swelling or angioedema-related concept if supported	Do not automatically call it angioedema when the source only states swelling.
Hypersensitivity-type pattern	May be represented through manifestations and/or supported diagnosis	Do not replace the narrative with an unsupported diagnosis.

Why codes are not invented here

The task materials do not provide a MedDRA version or coding convention. This section therefore demonstrates the coding approach without fabricating codes.

Narrative versus coding

The narrative remains the source description; structured coding should improve consistency and retrieval without changing the meaning of the reported event.

13. ICSR data-quality and follow-up plan

Open item	Why it matters	Follow-up action
Exact treatment dates	Precise chronology and duplicate review.	Obtain verified calendar dates.
Exact onset/recovery dates	Supports temporal and outcome analysis.	Request verified dates.
Route/formulation	Completes exposure details.	Confirm from source.
Patient weight	Dose/context completeness.	Retrieve if recorded and relevant.
Allergy history	Clinical interpretation.	Check authorized history.
Investigations	May strengthen event characterization.	Add only verified results.
Management/treatment	Supports seriousness/outcome.	Confirm any symptomatic or emergency treatment.
Reporter/source contact	Case completeness.	Obtain through authorized source.
Product identifiers	Traceability and duplicate checking.	Verify product/manufacture if available.

Follow-up priority

Highest-value follow-up items are those that could change causality, seriousness, expectedness or case validity. Missing information should be requested, not invented.

Duplicate-control principle

A production safety database would compare patient, event timing, product exposure, reporter/source and narrative characteristics. The current educational case lacks sufficient identifiers for a true duplicate assessment.

14. Drug Safety Associate workflow simulation

Stage	Action in this simulated case	Expected output
1. Intake	Receive case and identify potential ADR information.	Case record created.
2. Triage	Review for serious outcomes and urgent clinical information.	Non-serious training classification.
3. Data entry	Capture patient, product, event, timeline, action and outcome.	Structured case fields.
4. Narrative	Convert source facts into chronological safety narrative.	ICSR-ready narrative.
5. Coding	Apply approved coding practice.	MedDRA-oriented event terms; validated tools required in real work.
6. Causality	Review timing, dechallenge, rechallenge and alternatives.	Probable / Likely educational conclusion.
7. Seriousness	Apply outcome-based seriousness criteria.	Non-serious educational classification.
8. Expectedness	Compare against current product RSI.	Expected for training; real RSI verification required.
9. QC	Check contradictions, missing data and unsupported claims.	Traceable case.
10. Follow-up	Request missing source information as appropriate.	Improved completeness.
11. Medical review	Escalate clinical questions as applicable.	Documented reviewer assessment.
12. Closure	Ensure required information and rationale are recorded.	Complete educational package.

Operational learning point

The workflow links Tasks 1-4 into an end-to-end case-processing sequence: identify -> document -> assess causality -> classify -> quality-check -> follow up -> close.

15. Final ICSR assessment and portfolio checklist

Domain	Final educational assessment
ADR identification	Suspected ibuprofen-associated hypersensitivity-type reaction involving pruritic raised rash and mild lip swelling.
Suspected medicine	Ibuprofen 400 mg three times daily after food, newly initiated for bacterial pharyngitis.
Temporal relationship	Compatible; reaction on Day 2 after initiation.
Dechallenge	Supportive; symptoms improve within approximately 24 hours after withdrawal.
Rechallenge	Not performed.
Alternative causes	No new food, cosmetic, insect exposure or other newly started medicine reported.
Causality	Probable / Likely in the Task 3 educational framework.
Seriousness	Non-serious in the Task 4 educational framework.
Expectedness	Expected for the Task 4 training classification; verify current product RSI in real work.
Clinical severity	Not formally graded in source; educational characterization is mild-to-moderate symptom burden without documented systemic compromise.
Case completeness	Educationally sufficient for integrated assessment; incomplete for a real regulatory submission because several fields are unavailable.

Final portfolio checklist

Check	Status
Task 1 ADR identification carried forward	Complete
Task 2 reporting/documentation carried forward	Complete
Task 3 causality conclusion carried forward	Complete
Task 4 seriousness/expectedness carried forward	Complete
Source gaps explicitly marked	Complete
No unsupported patient identifiers/results invented	Complete
Chronological ICSR narrative	Complete
Drug Safety Associate workflow demonstrated	Complete
Fictional educational disclaimer included	Complete

Final declaration: This 16-page Task 5 document is a fictional educational ICSR simulation prepared for a pharmacovigilance internship. It is not a real safety report, medical advice or regulatory submission. Real ICSR processing must use verified source information and current applicable procedures.

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